

Healthcare Facilities & Managed Care

Healthcare Horizons: The First Baby Boomer Hits 80 This Year, So What?

With this research note, we are introducing a new thematic report which we are calling "Healthcare Horizons." We plan to use these reports to look at broader evolving themes in healthcare services. This first edition examines the implications of the first baby boomer reaching 80 years of age in 2026.

The Baby Boomer Demographic is Set to Boost the "Over 80" Age Cohort for the Next 15 Years

The year 2026 represents a pivotal demographic milestone, as the first members of the Baby Boomer generation (born 1946–1964) reach 80 years of age. Although population aging has been an enduring trend, numerous demographic analyses highlight that outcomes, healthcare utilization, and expenditures undergo significant shifts once individuals reach their 80s, setting this stage apart from the broader 65+ demographic. Brookings Institution research [projects](#) that the U.S. population aged 80 and older will approximately double between the mid 2020s and mid 2040s, rising from roughly 15 mln to nearly 30 mln individuals. U.S. Census Bureau data similarly shows older population cohorts are expected to grow at a faster rate than younger age cohorts. More specifically, the Census Bureau projects that adults aged 85 and older are expected to reach ~14 mln by 2040, representing a 72% increase over the roughly 6 mln adults aged 85 older as of 2022. From the standpoint of healthcare services, this transition is significant not solely due to population expansion, but also because medical care requirements, length of care, and dependence on support services escalate considerably beyond age 80, altering both the composition and magnitude of healthcare demand.

Healthcare Spending Increases Significantly with Age

National Health Expenditure data shows that healthcare costs increase significantly with age and escalate further among the oldest age cohorts. For instance, in 2020, personal healthcare spending per capita for individuals aged 65 and older amounted to \$22,356, which is over five times the spending level for children and approximately 2.5 times that of working-age adults. Despite representing just 17% of the population, this age group accounted for ~37% of total healthcare expenditures in 2020. CMS data also reveals that per capita healthcare spending for adults aged 85 and older is even higher, averaging around \$36,000, exceeding the spending levels for younger seniors. This data underscores the notion that the demographic shift toward an aging population, particularly those aged 80 and above, creates a structural tailwind for healthcare spending, which is anticipated to further accelerate moving forward.

However, The Type of Care Healthcare Utilized Changes with Increasing Age, Particularly at the 80+ Mark

As the population ages from 65 toward 80+, healthcare utilization shifts meaningfully from preventative and elective services toward more complex chronic disease management and higher-acuity care, driven by rising multi-morbidity and greater care coordination needs across settings. While younger seniors (65–74) remain in a "maintenance phase," with spend concentrated in outpatient, physician, and drug utilization, the 75–84 cohort sees a notable step-up in inpatient admissions, specialist intensity, and post-acute utilization. This trend accelerates further in the 85+ population, where spending increasingly skews toward duration-based services such as skilled nursing, home health, hospice, and long-term services and supports (LTSS). Against this backdrop, aging demographics—already consuming healthcare at a multiple of the general population—should continue to drive incremental demand across the system, even as care delivery structurally shifts. Concurrently, a growing preference for "aging in

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place” (with a majority of seniors favoring home-based care) reinforces a secular tailwind for home health and community-based services, supported by cost considerations, social factors, and improved at-home care capabilities.

SNFs, Home Health, Inpt Rehab, Hospice, Assisted Living, Funeral Homes To See a Positive Inflection Point

We expect post-acute settings to be a primary beneficiary of the growth in the over 80 population. This includes settings such as inpatient rehab facilities (IRFs), home health care, hospice, and skilled nursing facilities (SNFs), which we would expect to see volume benefits from the growth of this population, as these settings primarily care for an older patient cohort. Volumes to lower cost sites of care should also benefit from the fact that MCOs and payers are increasingly looking to control costs and, as such, likely to steer patients to these lower cost settings, such as home health care and skilled nursing facilities (SNFs). As such, we view home health, IRF, and skilled nursing operators as clear winners from this demographic trend. Areas like assisted living, hospice, personal care, and funeral homes are also likely to benefit. As a result, we are highlighting seven names we believe could benefit from the aging baby boomers over the next decade: **Addus HomeCare Corporation (ADUS), BrightSpring Health Services (BTSG), Encompass Health (EHC), Ensign Group (ENSG), Health Care Services Group (HCSG), PACS Group (PACS) and Service Corporation (SCI). Non-covered stocks exposed to the theme include Brookdale Senior Living (BKD), Chemed (CHE) and Pennant Group (PNTG).**

The UBS HOLT Lens Provides Further Insight

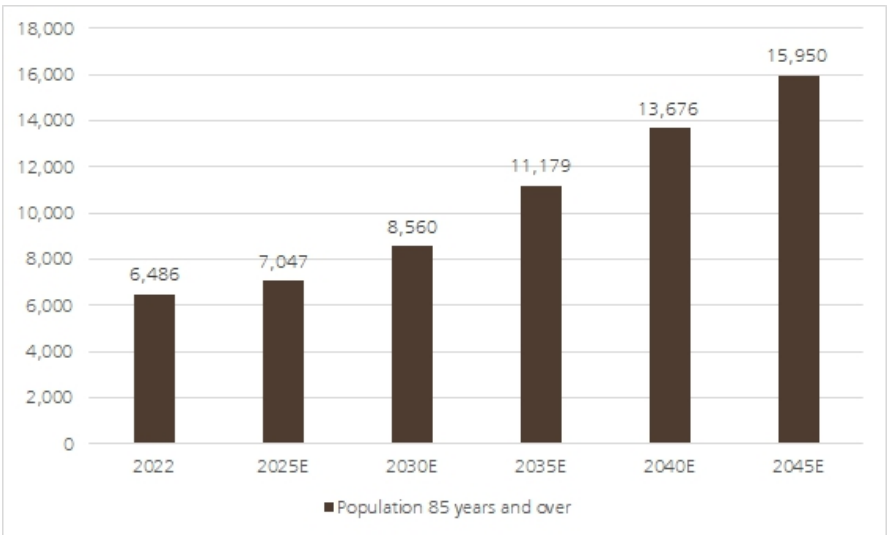
In this report, we also leverage the UBS HOLT framework to supplement our analysis. UBS HOLT is an objective, cash flow-based framework for evaluating corporate performance and valuation that is independent of our fundamental analysis. Based on the HOLT Scorecard, this group screens favorably overall, with eight of ten companies ranking in the top half of the HOLT universe. Operational Quality is the strongest factor, with five firms in the top quintile (ADUS, BTSG, CHE, EHC, and SCI) supported by their high and stable Cash Flow Return on Investment (CFROI) profiles.

ASSESSING THE IMPACT OF AN AGING POPULATION

U.S. Population is Getting Older

The year 2026 marks a demographic inflection, as the first members of the Baby Boomer cohort (born 1946–1964) reach age 80. Although population aging has been a persistent trend, numerous demographic analyses highlight that outcomes, healthcare usage, and expenditures undergo significant shifts when individuals reach their 80s, setting this stage apart from the wider 65+ demographic. Brookings Institution research [projects](#) that the U.S. population aged 80 and older will approximately double between the mid 2020s and mid 2040s, rising from roughly 15 million to nearly 30 mln individuals. U.S. Census Bureau data similarly shows older population cohorts are expected to grow at a faster rate than younger age cohorts. More specifically, the Census Bureau projects that adults aged 85 and older are expected to reach ~14 mln by 2040, representing a 72% increase over the roughly 6 mln adults aged 85 older as of 2022. From a healthcare services perspective, this shift is important not simply because of population growth, but because care needs, duration of care, and reliance on support services increase disproportionately after age 80, changing both the mix and intensity of healthcare demand.

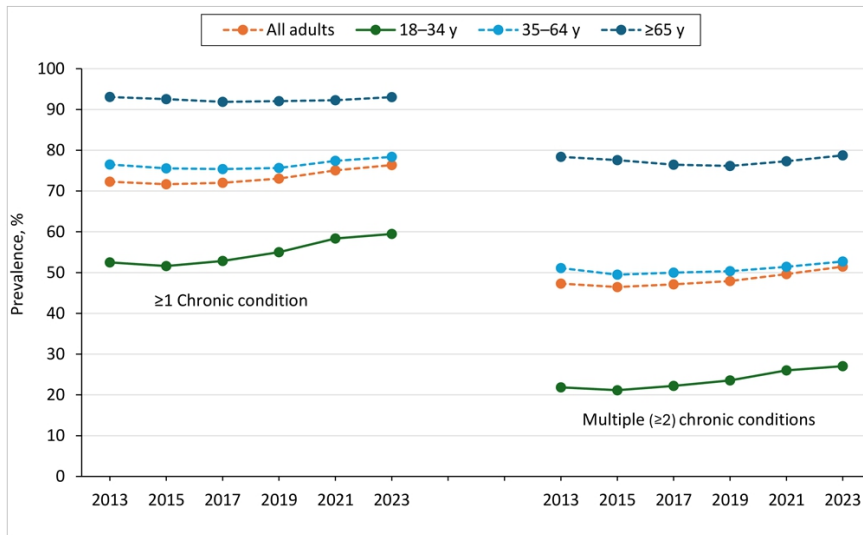
Figure 1: Adults in the U.S. Aged 85 Years and Older, in thousands



Source: Census Bureau and UBS Research; population in thousands

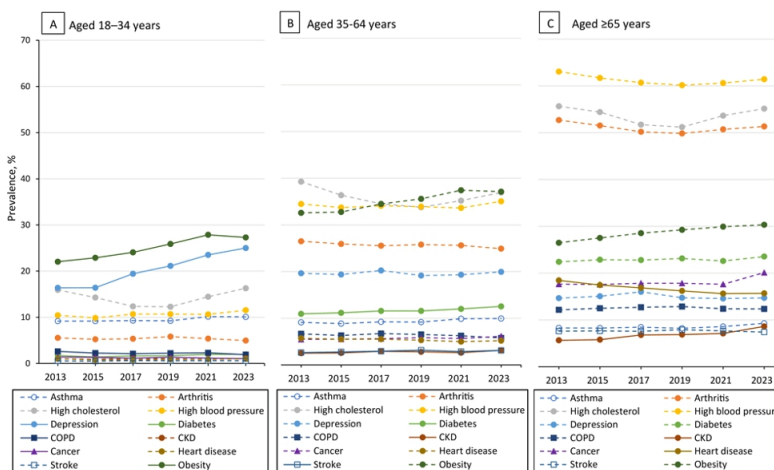
The U.S. population is also expected to see an increasing rate of chronic conditions among the population. Per Frontier Health, the number of people aged 50 years and older with at least one chronic disease is estimated to increase by 99.5% (to ~143 mln from ~72 mln in 2020) by 2050, while those with multi-morbidities are forecasted to increase just over 90% to 15 mln by 2050. Data from the Centers for Disease Control and Prevention found that 9 in 10 older U.S. adults report 1 or more chronic conditions, while ~79% of older adults had multiple chronic conditions.

Figure 2: Prevalence of Chronic Conditions Increases with Age



Source: [Centers for Disease Control](#)

Figure 3: Trends in Chronic Conditions



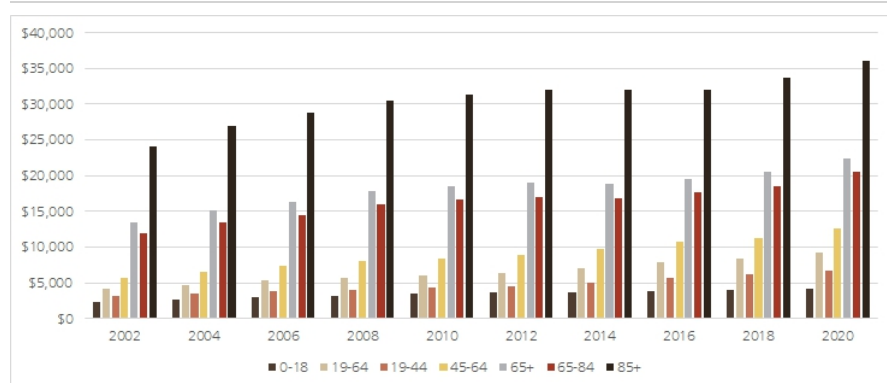
Source: [Centers for Disease Control](#)

Spending Rises Sharply with Advanced Age

With the increasing prevalence of chronic conditions, frailty, need for daily living assistance, etc., it is no surprise that healthcare spending increases with age. National Health Expenditure [data](#) provides clear evidence that healthcare spending rises steeply with age and accelerates further at the oldest ages. For example, personal healthcare spending for individuals aged 65 and older totaled \$22,356 (per capita) in 2020, more than five times the level for children and roughly 2.5 times that of working age adults. The age cohort also accounted for the smallest portion of the population (e.g. 17%), but accounted for ~37% of all healthcare spending in 2020. CMS tables show that per capita healthcare spending for adults aged 85 and older is even more elevated, reaching ~\$36,000, substantially higher than for younger seniors. Although individuals 85+ represent a relatively small share of the total population, they account for a disproportionately large share of total personal healthcare expenditures. This data supports the premise that the aging of the population into the 80+ cohort creates a structural spending tailwind, which we expect to accelerate moving forward. While not the only contributor to expectations for increased healthcare costs in the U.S., we do

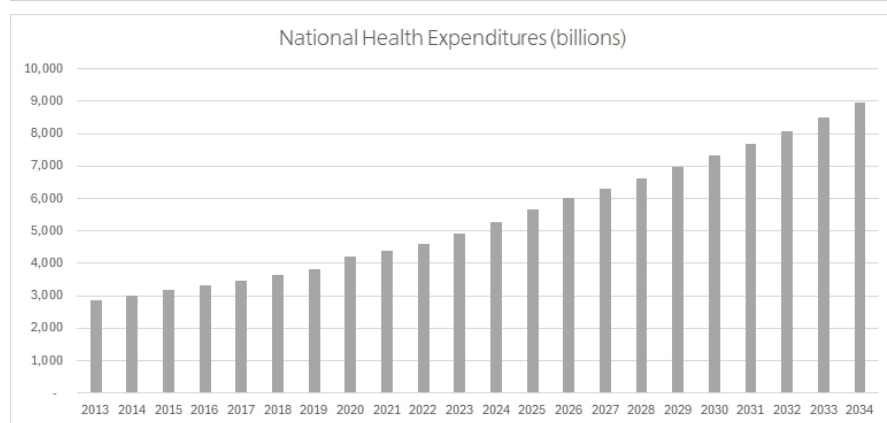
believe the aging baby boomers are one of the drivers of the growing total of healthcare expenditures in the country. CMS's National Healthcare Expenditure data forecasts healthcare expenditures growing to ~\$9 trillion by 2034, representing a 5.4% CAGR (2025-2034). CMS notes that this growth is projected to outpace that of average GDP growth (~4%), resulting in an increase in the health spending share of GDP from ~18% to ~21% by 2034.

Figure 4: Total Personal Health Care Per-capita Spending by Age Group



Source: Census Bureau and UBS Research

Figure 5: U.S. Healthcare Expenditure Projections



Source: CMS

Seniors Healthcare Utilization Changes as They Age

As seniors transition from age 65 toward 80 years of age, healthcare utilization shifts materially from preventative care and elective procedures toward the intensive management of chronic disease and higher-acuity treatment. This reflects the growing burden of multi-morbidity, which increases both the frequency and complexity of care encounters, driving higher utilization across primary, specialty, and coordinated care settings. Industry commentary underscores that aging cohorts require progressively greater chronic disease management and higher-intensity interventions, and this dynamic is amplified by the fact that individuals aged 65+ already utilize healthcare services at a multiple of the general population. As a result, the aging demographic will only serve to increase demand of healthcare resources and services in the U.S. At the same time, care delivery continues to migrate toward outpatient and ambulatory settings in earlier stages of the aging curve, reflecting both clinical preference and improved outcomes in lower-acuity environments.

Importantly, the increase in spending with age is not purely a volume story but reflects a fundamental rotation in the mix of care delivered. In the 65–74 cohort, spending is still dominated by outpatient, physician, and drug utilization consistent with a “maintenance phase” of chronic disease management, with limited reliance on institutional care. By ages 75–84, rising multi-morbidity drives a step-up in inpatient

admissions and specialist intensity, alongside a growing role for post-acute services as more patients are discharged to rehabilitation and skilled nursing settings. The most pronounced shift occurs in the 85+ cohort, where spending becomes increasingly concentrated in post-acute and supportive care categories, including skilled nursing facilities (SNFs), home health, and hospice. At this stage, cost growth is driven less by discrete clinical interventions and more by duration-based utilization, including SNF days, home health episodes, and end-of-life care, as well as long-term services and supports (LTSS).

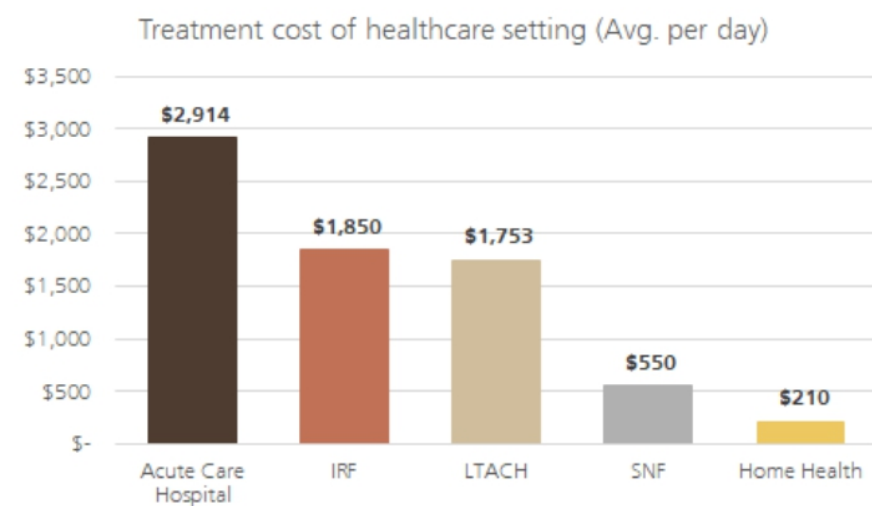
IMPLICATIONS FOR HEALTHCARE SERVICES

Site of Care Shifts to Benefit Post-Acute Settings

We expect post-acute settings to be a primary beneficiary of an aging population. This includes settings such as inpatient rehab facilities (IRFs), home health care, hospice, and skilled nursing facilities (SNFs), which we expect to see volume benefits from an aging population, as these settings primarily care for an older patient cohort. Volumes to lower cost sites of care should also benefit from the fact that MCOs and payers are increasingly looking to control costs and, as such, likely to steer patients to these lower cost settings.

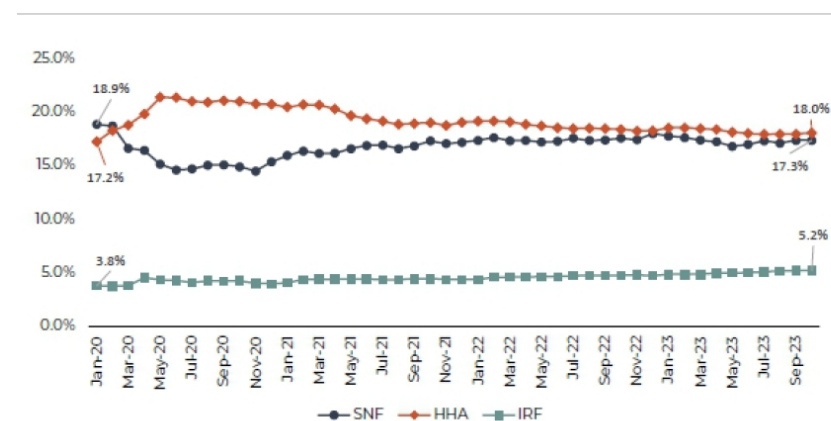
Home Health viewed as one of the biggest beneficiaries: As MCOs and state/federal funded agencies look to optimize treatment and cost efficiency, home-based care have emerged as beneficiaries given their lower cost. As seen below, home health care is up to 13x cheaper than the most expensive treatment solutions. Furthermore, research suggests that the benefits to home-based care are not just financial. Indeed, home based care has been shown to be less costly and importantly result in fewer readmissions relative to hospital care according to a study published in the National Library of Medicine (see [here](#)). In addition, the National Library of Medicine estimates that there is roughly \$41 bln in annual avoidable costs related to patients readmitted within 30 days after discharge. Furthermore, a study published in the American Journal of Managed Care found that patients who received home health care had a 60% lower risk of readmission.

Figure 6: Per Day Treatment Cost based on Site of Care Setting



Source: Company reports and Genworth Cost of Care Survey

Figure 7: Share of Post-Acute Discharges for Medicare Fee-for-Service



Source: MedPac Databook

Acute Hospital Care at Home episodes were also associated with lower Medicare spending in the 30-day post-discharge period across more than half of the top 25 Medicare Severity Diagnosis Related Groups (MS-DRGs) compared to traditional inpatient episodes. All this goes to support the cost-efficiency and increased attractiveness of home-based care for payors, patients, and providers. It is unsurprising, therefore, that McKinsey [estimates](#) up to \$265 bln worth of care currently delivered in traditional facilities for FFS and MA beneficiaries could shift to the home.

Not only is the US population growing older, but seniors increasingly show a preference to stay home and receive care in what is coined "aging in place". AARP's latest Home and Community Preferences [Survey](#) showed that older adults (60+) overwhelmingly want to stay in their homes (75%) and communities (73%). Reasons for this renewed desire to age in place include, but are not limited to, cost of living, maintaining existing social and familial connections, confidence in new assistive technology (doorbells, cameras, smart home devices etc.). While many (44%) feel an institutional move away from home is inevitable (mostly due to health concerns), the growing desire for at home health and aging is yet another tailwind for home health settings.

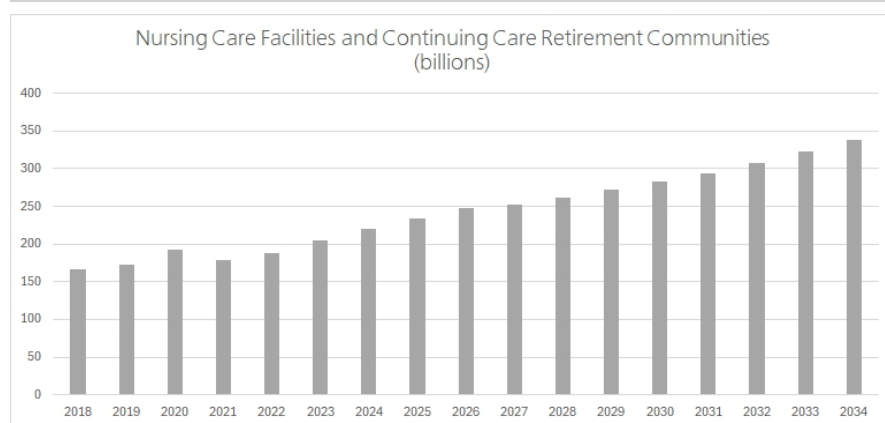
Given the expectations for increasing volumes moving towards home health settings, it is no surprise that CMS's NHE projections show healthcare expenditures related to home health care growing at a faster rate than total healthcare expenditures, with NHE forecasting home care growing ~7.8% CAGR (from 2025-2034). This is above the ~5.4% CAGR for total healthcare expenditures and the ~4.8% CAGR for hospitals, and ~4% for nursing care facilities and continuing care retirement facilities.

Personal Care Services: While many seniors remain active at 65, the demand for help with activities of daily living (ADLs) increases significantly by age 80. According to Addus, approximately 90% of adults aged 65 and older prefer to "age in place" for as long as possible. Personal care services are able to meet this demand by providing non-medical assistance with tasks such as bathing, dressing, meal preparation, and housekeeping, which are essential for seniors who are at increased risk of hospitalization or institutionalization. The need for assistance with ADLs increases with age, and with the increasing volume of older seniors, the demand for services to assist with ADLs will only increase. According to a 2018 [study](#), roughly 20% of adults aged 85 or older needed help with ADLs, which is more than double the 8.7% of adults aged 75-84 that needed help with ADLs and compares to the 3.7% for the 65-74 age group.

Skilled Nursing Facilities Another Low Cost Site of Care Option

Similar to home health, we see Skilled Nursing Facilities (SNFs) as one of the main beneficiaries of the aging population trends. Industry operators, such as ENSG, are looking for this to be a decade long tailwind in which it is just in the early innings. While not as cheap as home health, SNFs do offer a low cost site of care setting for payers at just \$550 per day. In addition, top operators who exhibit high quality scores are also able to produce quality outcomes for their patients, and would continue to be a discharge site of care of choice for referral managers.

Figure 8: NHE Expenditure Forecasts for Nursing Care Facilities and Continuing Care Retirement Communities (millions)



Source: CMS

Hospice Volumes Should Also Benefit from Aging Demographics

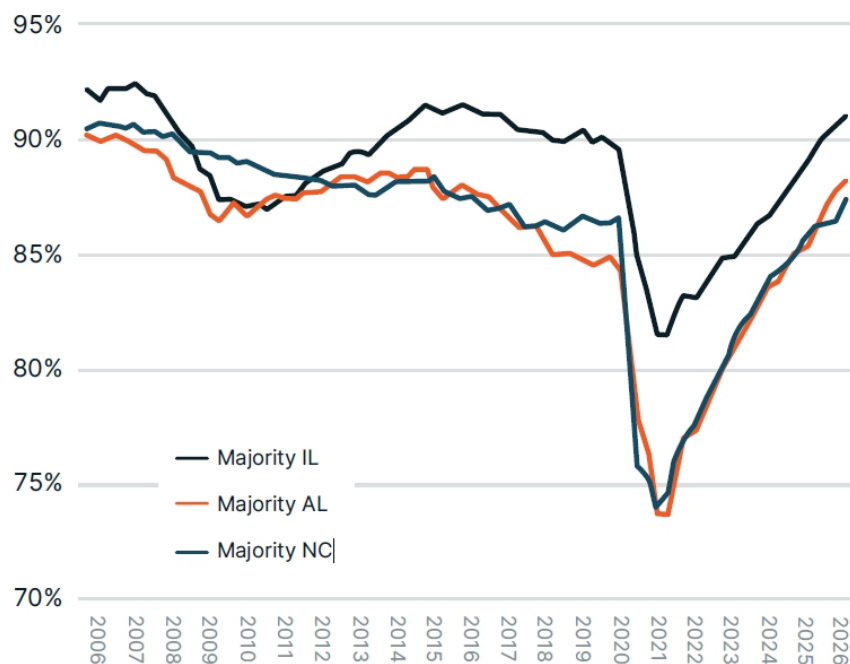
Hospice volumes benefit from significant structural support driven by demographic aging, as the demand for end-of-life care is closely tied to age and increases disproportionately within the 75+ age group. With rising life expectancy, a larger portion of the population reaches advanced age categories where the prevalence of chronic, progressive conditions—such as cancer, dementia, and cardiovascular disease—escalates, expanding the pool of patients eligible for hospice services. According to the [Caring Hospice Institute](#), the average age at hospice admission is ~80 years old, and the largest single age bracket is 85 and older. From a volume perspective, the growth of the elderly population not only increases the total number of decedents but also extends the duration of functional decline prior to death, supporting earlier referrals and longer lengths of stay, which collectively drive total patient days. Additionally, heightened awareness, evolving physician referral practices, and Medicare support have contributed to steadily increasing hospice utilization rates, indicating that demographic expansion is being complemented by secular penetration gains. According to the [National Alliance for Care at Home](#), 1.9 mln Medicare beneficiaries were enrolled in hospice care for one day or more in CY24, representing a 4.4% increase versus CY23. In addition, of all Medicare decedents in CY24, 53.1% received at least one day of hospice care and were enrolled in hospice at the time of death, and marks the third year of improvement.

Senior Housing and Assisted Living: Demand Rising, Supply Constrained

Senior housing data similarly reflect rising demand aligned with demographic aging. The National Investment Center for Seniors Housing & Care (NIC) reports that U.S. senior housing occupancy reached approximately 88–89% in 2025, marking more than four consecutive years of quarterly occupancy increases. Data as of 1Q26 from NIC, shows occupancy rates at 89.5%, up 40 bps Q/Q. NIC has noted that the strengthening demand is being driven by the aging Baby Boomer population, with limited supply of new inventory, with growth in inventory below 1% in 4Q25 and marked the third consecutive quarter of less than 1% growth in inventory. NIC [projects](#) that senior housing occupancy rates could exceed 90% by the end of 2026, particularly in independent and assisted living categories, as demand continues to outpace new construction. We look for demand for senior housing and assisted living to only increase from here given the demographic trends expected in the U.S.

Figure 9: Occupancy by Property Type

NIC MAP | Primary Markets



Source: NIC MAP: IL - Independent Living; AL - Assisted Living; NC - Nursing Care

Deathcare Volumes to also Benefit from Aging Demographics

The aging of the Baby Boomer cohort into advanced ages is also relevant for deathcare services, including funeral homes, cemeteries, and cremation providers. Mortality rises sharply with age, particularly after age 80, making this demographic milestone a positive volume driver over time. In fact, SCI, one of the largest funeral and cemetery operators in the U.S. has noted that we are only in the early innings of a decade long expected tailwind from the aging Baby Boomer population. U.S. life tables from the CDC and Social Security Administration show that annual mortality probabilities increase exponentially in advanced age cohorts, despite incremental improvements in life expectancy at older ages. The National Funeral Directors Association (NFDA) [projections](#) call for the number of deaths to rise through 2045, anticipating a 19% increase by 2045 compared to 2022. That said, revenue growth is moderated somewhat by continued shifts in consumer preferences. NFDA forecasts cremation rates to increase significantly, reaching 82.3% by 2045, from ~63% in 2025, while burial rates are expected to fall to 13.3% by 2045. Cremation services generally carry lower average revenue per service than traditional burial, creating mix pressure for at need funeral revenues even as volumes rise. NFDA notes that families' increased preference for cremation is influenced by cost considerations, environmental concerns, fewer religious prohibitions and a desire for simpler funeral ceremonies, while the increase in the number of individuals without a religious affiliation has also contributed to the decline in traditional funerals and the rise in cremation. Cemetery operators and pre-need businesses are also expected to benefit from an aging population. Pre-need sales increase with age, as individuals entering their 70s and 80s are more likely to prearrange funeral and cemetery services. These contracts generate deferred but relatively predictable future revenue streams.

Hospitals

While we have pointed out the fact that the aging demographic shifts will be beneficial for post-acute settings, we think hospitals will see a benefit as well from this trend. Hospitals are expected to benefit from population aging as older patients drive disproportionately higher acute care utilization, particularly in the 75+ and 80+ cohorts. Utilization data consistently show that the 65+ population uses hospital services at materially higher rates than younger adults, reflecting both a higher burden of chronic

disease and greater incidence of acute events (e.g., cardiovascular episodes, infections, falls). For example, according to [Vizient](#), 72% of emergency department visits in 2024 for adults aged 65+ were classified as emergent, higher than the 65% rate for adults aged 18-64. Over the next decade, Vizient projects emergent visit volume for the 65 and older population cohort to grow by 28%, while the 75-84 year old age group is expected to see a 45% increase. As the population ages, this should translate into continued growth in inpatient admissions, emergency department visits, and overall hospital volumes. In addition, the expansion of the Medicare-eligible population—particularly the continued growth of Medicare Advantage—provides a large and relatively stable payer base tied to aging demographics. According to projections by [Vizient](#), volumes are expected to grow across all sites of care for the 65+ age cohort. This includes an expected 24% increase (from 2024-2034) in emergency department volumes for the 65+ population, a 17% increase in inpatient discharges, 22% increase in inpatient days, and 30% increase in outpatient volume. For reference, these are all below the 32% increase expected by Vizient for post-acute care volumes.

Interestingly, hospital inpatient volumes do not see any material decline in utilization as seniors age. According to CMS data on Medicare Fee-for-Service hospital inpatient stays, the discharges per 1,000 Medicare Part A enrollees is the lowest for the 65-74 age cohort at 147 discharges, then increases to 275 for 75-84 year olds and then again to 461 for the 85-94 year old cohort.

Figure 10: Medicare Fee-for-Service Inpatient Hospital Utilization Trends by Age Cohort

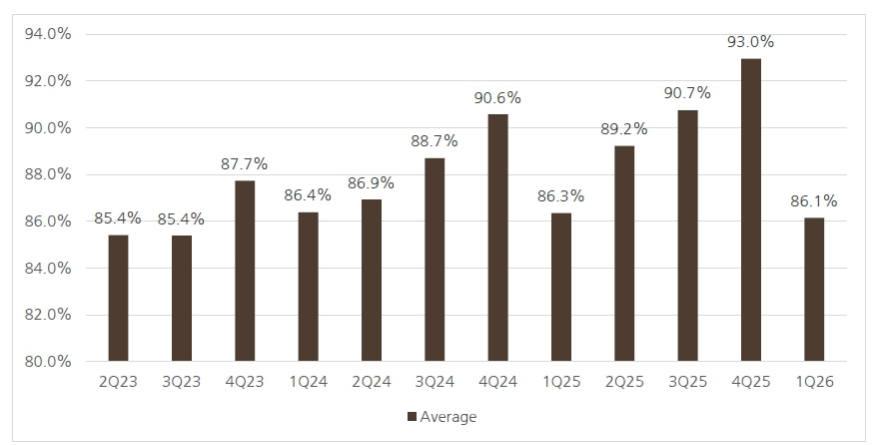
Age Cohort	Total Original Medicare Part A Enrollees	Total Persons With Utilization	Total Discharges	% of Total Discharges	Discharges Per 1,000 Original Medicare Part A Enrollees	Discharges Per 1,000 Persons With Utilization	Total Program Payments	% of Total Program Payments	Program Payments per Original Medicare Part A Enrollee
65-74 Years	17,223,233	1,597,656	2,539,369	31.9	147	1,589	43,281,196,154	34	2,513
75-84 Years	9,666,928	1,619,019	2,657,792	33.4	275	1,642	42,922,968,459	34	4,440
85-94 Years	3,188,185	901,809	1,470,759	18.5	461	1,631	20,651,899,535	16	6,478
95 Years and Over	447,894	143,316	217,759	2.7	486	1,519	2,729,881,221	2	6,095

Source: CMS and UBS Research

MCOs Could See Pressure from Rise in Senior Member Utilization, but Look for MCOs to Steer Members to Lower Cost Settings

The aging population in the U.S. has already served to benefit Managed Care Organization's (MCOs) Medicare Advantage (MA) membership totals, which in turn has helped grow premiums. With the increasing age of MA members, there could in theory be increased utilization as members encounter incremental medical episodes given the increasing prevalence of chronic diseases amongst seniors. However, it is important to realize that members with higher disease burdens generally come with higher risk scores, and therefore higher payments to MCOs from CMS to care for these sicker patients. As it relates to the risk model and age cohorts, there is a natural step up in the CMS-HCC model coefficient based on age. However, the age groupings are in 4-year increments (75-79, 80-84, 85-89, etc). As such, assuming a member that has unchanged HCC codes but sees a change in age to the next age bracket, this would create a tailwind to revenue, all else equal. With that, if the health status of the member were to deteriorate quickly after hitting 80, the MCO would not be able to capture the codes until the following year. This in turn would cause a lower revenue level and higher medical cost expense until the member's risk score is appropriately captured and scored. We believe MCOs will look to better manage this aging population cohort through things like greater utilization of value-based care contracts and looking to send members to lower cost site of care settings, such as home health.

Figure 11: MCO's MLR Trends



Source: Company Reports; excludes ALHC

Workforce and Caregiver Constraints as a Limiting Factor

Although demographic trends indicate rising demand, growth in volumes is constrained by workforce availability. According to the Bipartisan Policy Center, the U.S. has experienced a persistent shortage of direct care workers for over 20 years, with nearly 9 mln direct care job openings projected between 2022 and 2032. The report highlights that staffing shortages have compelled many home health providers to decline referred patients and have contributed to delays in hospital discharges. Concurrently, unpaid caregiving has emerged as a crucial component of the care system. AARP and the National Alliance for Caregiving's 2025 [report](#) estimates that 63 mln Americans provided unpaid caregiving, representing an almost 50% increase since 2015. Additionally, the Administration for Community Living observed that limited availability of direct care professionals restricts access to community-based services for older adults and individuals with disabilities. These trends point to a growing imbalance of demand for care given the aging population of the U.S., with a limited supply of caregivers to meet this demand.

LEVERAGING UBS HOLT TO FRAME THE DEBATE OVER OUR "OVER 80 BENEFICIARIES"

In this section, we leverage the UBS HOLT Framework to provide additional insights regarding the companies highlighted in this report. UBS HOLT is an objective, cash flow-based framework for evaluating corporate performance and valuation, separate from our work. At its core, HOLT measures returns on invested capital using its proprietary Cash Flow Return on Investment (CFROI) methodology, applied consistently across geographies and sectors. The framework also assesses valuation by modeling the sustainability and fade of a company's cash flow generation over time.

HOLT Scorecard and Factor Analysis

The UBS HOLT Scorecard is a systematic framework that ranks a company on three core dimensions: Operational Quality, Momentum and Valuation. These three pillars are weighted equally and combined into an Overall Scorecard Rank that helps investors identify attractive and unattractive stocks and has historically been associated with long-term shareholder returns. Based on HOLT's corporate performance and valuation metrics, the HOLT Scorecard provides a unique, differentiated and tractable factor set. Factors are ranked on a scale from 0 (weakest) to 100 (strongest) relative to a defined peer universe.

The **HOLT Operational Quality** factor assesses the relative attractiveness of a company based on its historical level and variability of CFROI. The **HOLT Momentum** factor provides a gauge of market sentiment, inclusive of 13-week CFROI Revisions, price momentum, and liquidity. The **HOLT Valuation** factor evaluates the relative attractiveness of a company's stock, and considers its HOLT Warranted Upside/Downside, Market Implied Yield, Economic P/E, HOLT P/B, and Dividend Yield.

[See here for more information on the HOLT Scorecard and Factor Library](#)

We include Descriptive Factor Peer Ranks, which do not contribute to the Overall Scorecard Rank but highlight firms that may warrant specific portfolio positioning based on their growth potential or defensive characteristics. The **HOLT Growth** factor gauges the extent to which a company is likely to generate higher or lower future cash flow growth relative to its peers. The **HOLT Low Volatility** factor measures the variability of a company's stock price and its sensitivity to movements in its regional market. To assess Low Volatility, the factor evaluates long-term monthly beta, long-term monthly price volatility, and 90-day price volatility.

Figure 12: Post-Acute Peers HOLT Scorecard and Factor Peer Rank

Symbol	BTSG	HCSG	ADUS	PACS	CHE	EHC	BKD	ENSG	SCI	PNTG
Overall Peer Rank	92	86	77	75	69	67	57	57	50	30
Operational Quality	94	78	93	68	98	81	34	78	81	72
Momentum	96	91	63	77	68	37	74	48	30	65
Valuation	8	25	29	45	9	49	60	37	36	6
Growth	92	78	90	96	31	61	12	78	38	75
Low Volatility	37	54	79	40	90	83	59	80	83	40

Source: UBS HOLT Lens. Data as of July 13, 2026. Overall Scorecard Peer Rank, Factor Peer Ranks, and Descriptive Peer Ranks derived on a Region basis. Rank is reflective of relative positioning within North America. Firms ordered left to right on HOLT Overall Scorecard Peer Rank.

In the figure above, we rank the Post-Acute peer group by Overall Scorecard Rank and its underlying factor components. The group screens favorably overall, with eight of ten companies ranking in the top half of the HOLT universe. Operational Quality is the strongest factor, with five firms in the top quintile (CHE, BTSG, ADUS, EHC, SCI) supported by high and stable CFROI profiles. Momentum is mixed, while valuation is generally less favorable, with all companies except BKD ranking in the lower half of the universe.

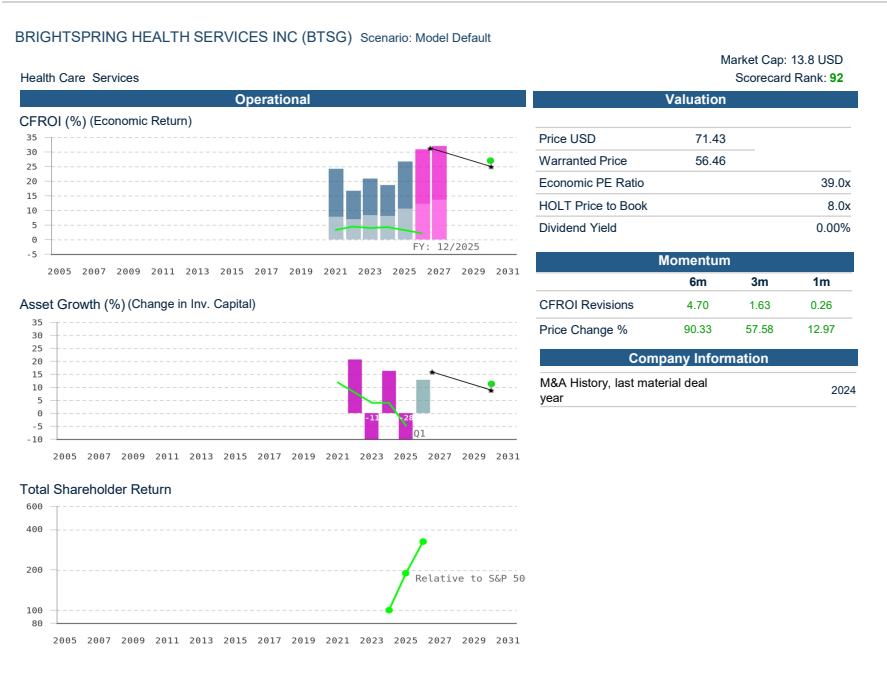
Individual Company Analysis in UBS HOLT Lens

In this section, we analyze the individual profiles of each of the Post-Acute peer group constituents through the UBS HOLT Lens. The analyses below are derived from the HOLT Model Default Scenario, which is based on HOLT's Systematic Fade Methodology. Investors are encouraged to override the default scenario in UBS HOLT Lens to reflect their own assumptions.

BrightSpring Health Services Inc (BTSG)

Among Post-Acute peers, BTSG earns the highest HOLT Overall Scorecard Rank of 92. Since its January 2024 IPO, shares of the home- and community-based health solutions provider have increased over 530% CFROI rose to a high of 27% in 2025, driven by increased asset turns as the firm rationalized its operating lease facilities and licenses through the divestiture of its Community Living business. Looking ahead, CFROI is forecast to increase further to 32% by 2027. However, the market appears to be discounting a reversion to 2025 CFROI levels, alongside mid-single-digit growth.

Figure 13: BTSG in UBS HOLT Lens

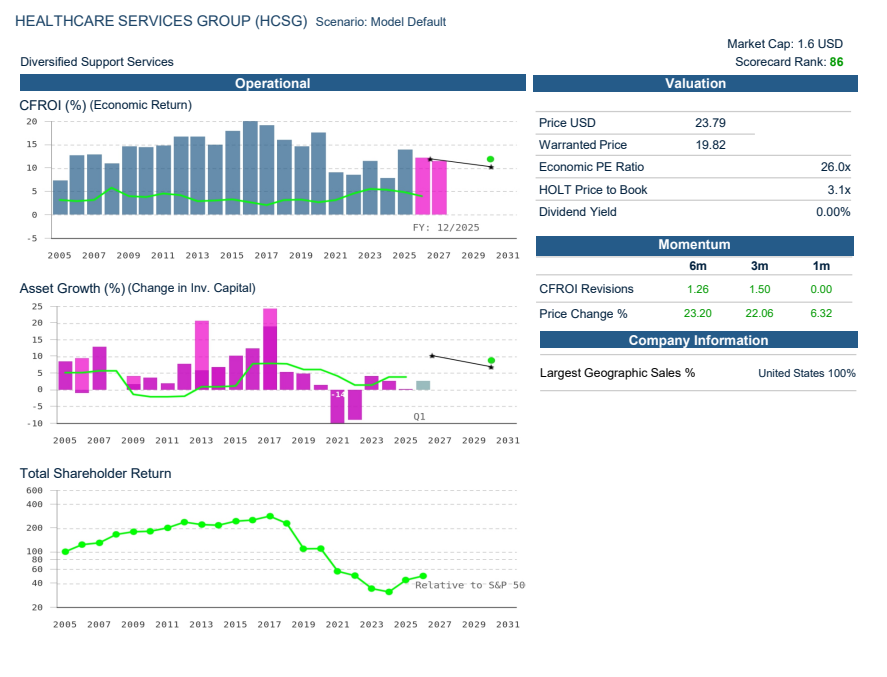


Source: UBS HOLT Lens. Data as of July 13, 2026.

Healthcare Services Group (HCSG)

HCSG's CFROI rebounded in 2025, driven by margin recovery from improved pricing and operating efficiencies, according to management. Strong share price performance and positive CFROI revisions following Q1 2026 have supported its top-decile Momentum factor. Looking ahead, CFROI is forecast to remain elevated, while the market is pricing in implied CFROI of approximately 12% and high-single-digit asset growth

Figure 14: HCSG in UBS HOLT Lens

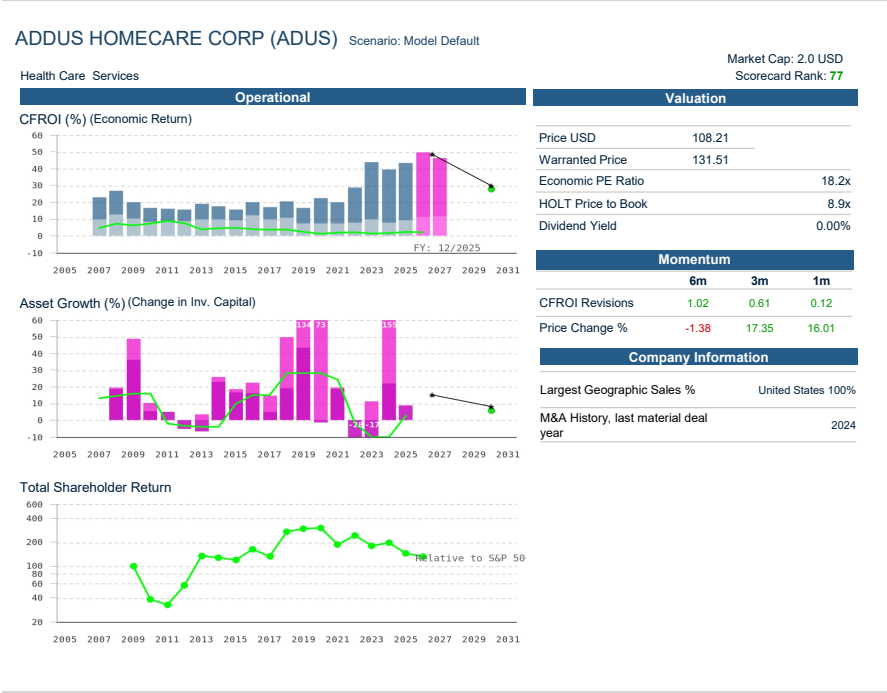


Source: UBS HOLT Lens. Data as of July 13, 2026.

Addus Homecare Corp (ADUS)

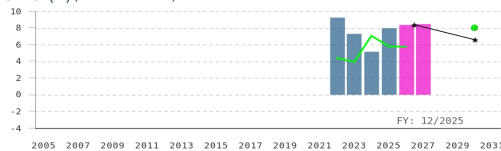
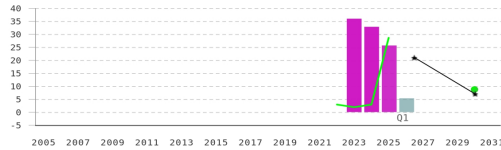
ADUS' CFROI has risen markedly over the past five years, supported by both margin expansion and higher asset turns. Notably, despite favorable CFROI revisions throughout much of the past year, the stock lagged the broader market for most of the period before its recent outperformance. Even after the rally, shares continue to trade below historical valuation norms, with Economic P/E of 18.2x versus a five-year median of 24.1x and HOLT Price-to-Book of 8.9x versus a historical median of 9.0x. Although consensus forecasts imply the company will continue to generate attractive returns, market-implied CFROI reflects skepticism about the durability of these levels, with current pricing suggesting CFROI will fade to roughly 28% over the long term.

Figure 15: ADUS in UBS HOLT Lens



Source: UBS HOLT Lens. Data as of July 13, 2026.

Strong share price performance and positive CFROI revisions support PACS' high HOLT Overall Scorecard Rank of 75. CFROI rebounded in 2025 as margins recovered following personnel investments that temporarily pressured SG&A expenses in 2024, causing CFROI to fall below the firm's cost of capital. CFROI is forecast to improve further over the next twelve months, while the market is pricing in stable CFROI levels alongside high-singledigit asset growth through 2030.

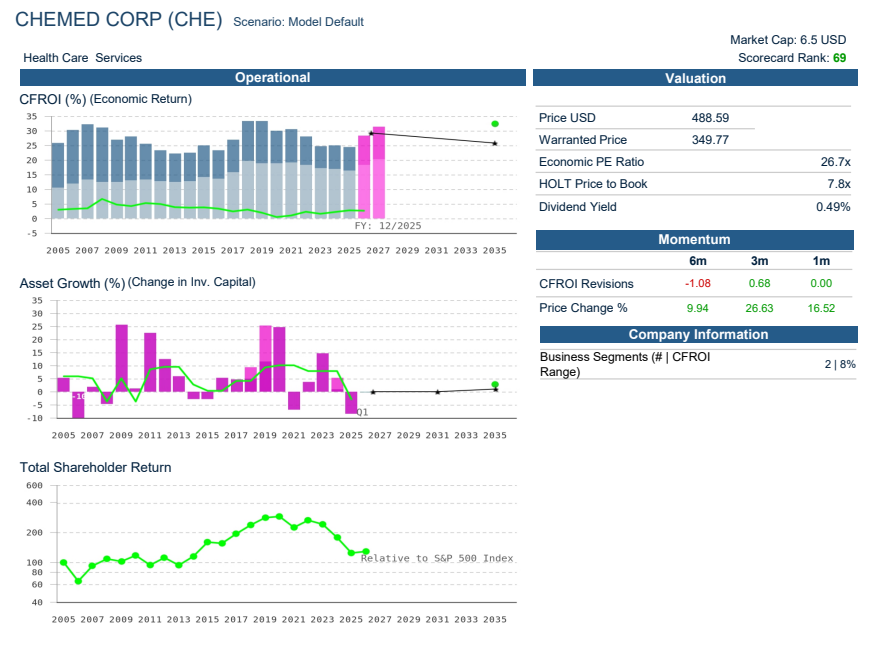
PACS GROUP INC (PACS)		Scenario: Model Default	Market Cap: 6.8 USD Scorecard Risk: 75
Health Care Facilities			
Operational		Valuation	
CFROI (%) (Economic Return)		Price USD43.22 Warranted Price30.96 Economic PE Ratio20.7x HOLT Price to Book1.7x Dividend Yield0.00%	
 FY: 12/2025		Momentum 6m 3m 1m CFROI Revisions0.380.15-0.31 Price Change %4.7228.0619.13	
Asset Growth (%) (Change in Inv. Capital)			
 01			
Total Shareholder Return			
 Relative to S&P 50			

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Chemed Corp (CHE)

CHE's CFROI is expected to improve over the next several years following a prolonged period of deterioration, driven more recently by weaker sales growth and margin pressure within its Roto-Rooter business. Looking ahead, company-wide CFROI is forecast to recover to 32% by 2027. Meanwhile, CHE's share price has rebounded from the multi-year lows reached earlier this spring, and market-implied CFROI suggests investors expect the company to sustain these higher near-term return levels over the long term, with modest asset growth.

Figure 17: CHE in UBS HOLT Lens

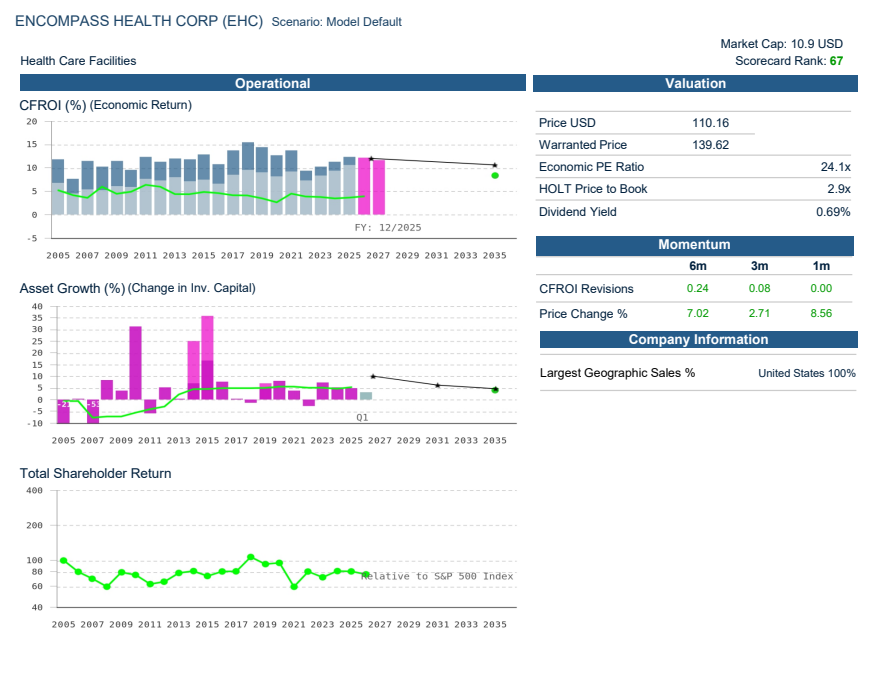


Source: UBS HOLT Lens. Data as of July 13, 2026.

Encompass Health Corp (EHC)

CFROI has inflected positively in recent years after a sharp drop in 2022 sales pressured asset turns, resulting in a decade-low CFROI that year. Over the time same period, margins have inflected positively, reaching a decade-high in 2025. Despite positive near-term price performance, shares are down on an absolute one-year basis, weighing on EHC's Momentum factor. Looking forward, CFROI is forecast to decline modestly by 70 bps through 2027, while the market is pricing for continued degradation longer-term.

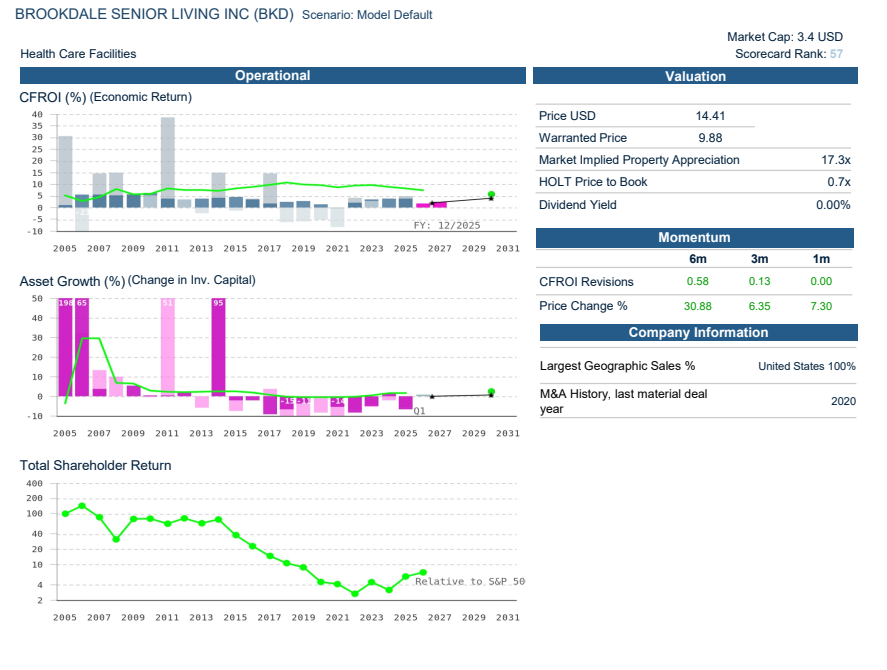
Figure 18: EHC in UBS HOLT Lens



Brookdale Senior Living Inc (BKD)

BKD, as an operator of senior living communities, is evaluated under HOLT's Real Estate model. The company's historically low CFROI profile has resulted in a depressed Operational Quality Score. In contrast, sentiment toward BKD has been favorable, with strong price performance and positive CFROI revisions driving the company into the second quartile of the Momentum factor. Although CFROI is forecast to decline over the next twelve months, market expectations imply a longer-term recovery toward historical levels.

Figure 19: BKD in UBS HOLT Lens



Source: UBS HOLT Lens. Data as of July 13, 2026.

ENSG ranks in the second quintile of HOLT's Overall Scorecard Rank, supported by strong Operational Quality and Momentum factors, though tempered by relatively less compelling valuation metrics. Over the past five years, the company has maintained stable CFROI levels above its cost of capital while generating high-single-digit asset growth. Looking ahead, consensus expectations point to continued CFROI stability, a view that is also embedded in current market-implied returns, suggesting investors broadly expect ENSG to sustain its existing economic profitability profile over the long term.

ENSGN GROUP INC (ENSG)		Scenario: Model Default	Market Cap: 9.8 USD																
Health Care Facilities		Scorecard Rank: 57																	
Operational		Valuation																	
CFROI (%) (Economic Return)		<table> <tr><td>Price USD</td><td>168.59</td></tr> <tr><td>Warranted Price</td><td>164.86</td></tr> <tr><td>Economic PE Ratio</td><td>29.0x</td></tr> <tr><td>HOLT Price to Book</td><td>2.5x</td></tr> <tr><td>Dividend Yield</td><td>0.15%</td></tr> </table>		Price USD	168.59	Warranted Price	164.86	Economic PE Ratio	29.0x	HOLT Price to Book	2.5x	Dividend Yield	0.15%						
Price USD	168.59																		
Warranted Price	164.86																		
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HOLT Price to Book	2.5x																		
Dividend Yield	0.15%																		
		<table> <tr><th colspan="4">Momentum</th></tr> <tr><th></th><th>6m</th><th>3m</th><th>1m</th></tr> <tr><td>CFROI Revisions</td><td>0.35</td><td>0.04</td><td>0.00</td></tr> <tr><td>Price Change %</td><td>-3.04</td><td>-15.29</td><td>12.87</td></tr> </table>		Momentum					6m	3m	1m	CFROI Revisions	0.35	0.04	0.00	Price Change %	-3.04	-15.29	12.87
Momentum																			
	6m	3m	1m																
CFROI Revisions	0.35	0.04	0.00																
Price Change %	-3.04	-15.29	12.87																
Asset Growth (%) (Change in Inv. Capital)		<table> <tr><th colspan="2">Company Information</th></tr> <tr><td>Largest Geographic Sales %</td><td>United States 100%</td></tr> </table>		Company Information		Largest Geographic Sales %	United States 100%												
Company Information																			
Largest Geographic Sales %	United States 100%																		
Total Shareholder Return																			

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Service Corp International (SCI)

Following its 2013 acquisition of Stewart Enterprises, SCI's CFROI improved steadily before peaking in 2021 on elevated COVID-related mortality. CFROI has since normalized around 12%, with the market pricing a gradual increase toward 14% over the long term. Despite strong Operational Quality, weak price momentum, negative CFROI revisions, and a premium valuation weigh on its overall HOLT Scorecard Rank, placing SCI in the bottom half of its peer group.

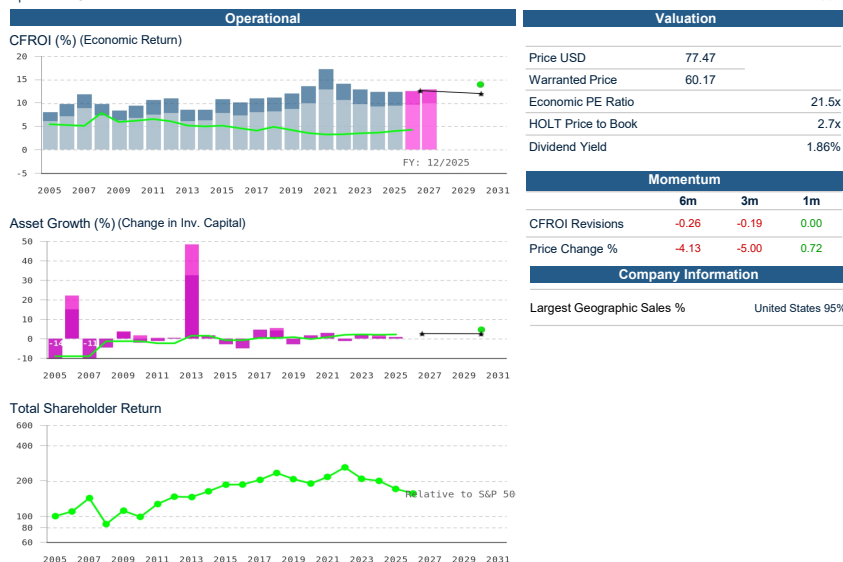
Figure 21: SCI in UBS HOLT Lens

SERVICE CORP INTERNATIONAL (SCI) Scenario: Model Default

Specialized Consumer Services

Market Cap: 10.8 USD

Scorecard Rank: 50

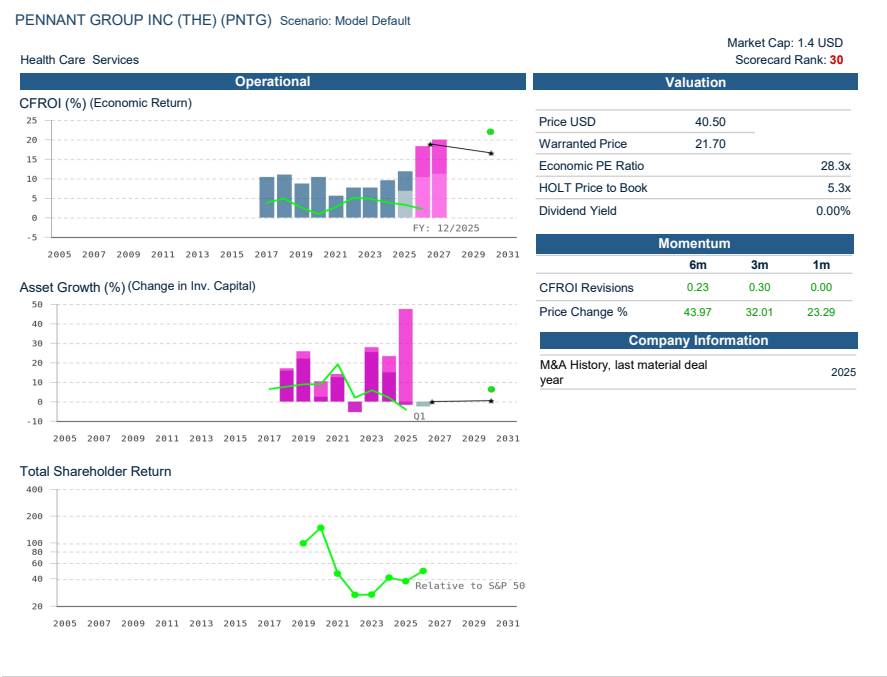


Source: UBS HOLT Lens. Data as of July 13, 2026.

The Pennant Group Inc (PNTG)

PNTG's \$146.5 million acquisition of UnitedHealth Group's home health and personal care services business in 2025 positions the company to benefit from the continued expansion of its home health, hospice, and senior living operations. Reflecting these growth opportunities, CFROI is forecast to increase from 12% in 2025 to 20% by 2027, while market-implied expectations suggest further expansion to approximately 22% over the longer term. Despite favorable rankings in Operational Quality and Momentum, elevated market expectations and premium HOLT valuation multiples place PNTG in the bottom decile of the Valuation factor.

Figure 22: PNTG in UBS HOLT Lens



Valuation Method and Risk Statement

Risk to the managed care group include higher than expected utilization, elevated disenrollment, and government regulation. We use a multiple based approach based on PE to arrive at our price targets for the managed care group. Risks to the provider sector include: 1) slower volumes, 2) deteriorating payer mix, 3) integration and execution issues, 4) increased government regulation, 5) higher-than-expected operational expenses, and 6) any unexpected adverse macro and regulatory developments. We use a multiple-based approach to arrive at our price targets.

The HOLT methodology does not assign ratings or a target price to a security. It is an analytical tool that involves use of a set of proprietary quantitative algorithms and warranted value calculations, collectively called the HOLT valuation model, that are consistently applied to all the companies included in its database. The HOLT valuation model is a discounted cash flow model. Third-party data (including consensus earnings estimates) are systematically translated into a number of default variables and incorporated into the algorithms available in the HOLT valuation model. The source financial statement, pricing, and earnings data provided by outside data vendors are subject to quality control and may also be adjusted to more closely measure the underlying economics of firm performance. These adjustments provide consistency when analyzing a single company across time, or analyzing multiple companies across industries or national borders.

The default scenario that is produced by the HOLT valuation model establishes a warranted price that represents the expected mean value for a security based upon empirically derived fade algorithms that forecast a firm's future return on capital and growth rates over an extended period of time. As the third-party data are updated, the warranted price updates automatically. A company's future achieved return on capital or growth rate may differ from HOLT default forecast. Additional information about the HOLT methodology is available upon request.

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HOLT transferred to UBS on October 1, 2024 – renamed as UBS HOLT. The UBS HOLT Lens website and materials will be transitioning to UBS branding over time, with expected full redesign to UBS in time.

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Buy	FSR is > 6% above the MRA.	55%	24%
Neutral	FSR is between -6% and 6% of the MRA.	40%	21%
Sell	FSR is > 6% below the MRA.	6%	21%

Source: UBS. Rating allocations are as of 30 June 2026.

1:Percentage of companies under coverage globally within the 12-month rating category.

2:Percentage of companies within the 12-month rating category for which investment banking (IB) services were provided within the past 12 months.

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UBS Securities LLC: AJ Rice, Ashley Minns, James Kurek, Jonathan Yong, Joseph Overman, Michael Wei, Steve Bock, CFA.

Company Disclosures

Company Name	Reuters	12-month rating	Price	Price date
Addus HomeCare Corp ^{16,28}	ADUS.O	Buy	US\$108.86	13 Jul 2026
BrightSpring Health Services ^{4,16,6,28}	BTSG.O	Buy	US\$69.64	13 Jul 2026
Encompass Health Corp ^{16,28}	EHC.N	Buy	US\$110.04	13 Jul 2026
Healthcare Services Group ¹⁶	HCSG.O	Buy	US\$24.04	13 Jul 2026
PACS Group Inc ¹⁶	PACS.N	Buy	US\$43.27	13 Jul 2026
Service Corporation International ^{16,28}	SCI.N	Buy	US\$79.64	13 Jul 2026
The Ensign Group Inc ^{16,28}	ENSG.O	Buy	US\$168.39	13 Jul 2026

Source: UBS Global Research; LSEG Eikon. All prices as of local market close. Ratings in this table are the most current published ratings prior to this report. They may be more recent than the stock pricing date.

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Additional Prices: Chemed Corp, US\$492.27 (13 Jul 2026); Brookdale Senior Living Inc, US\$14.40 (13 Jul 2026); Source: UBS. All prices as of local market close.

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